

Adolescence and Adolescent Health

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Health Promotion & Disease Prevention Course
(COMH 4012) for
Undergraduate Medical Students
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Learning Objectives

- Define adolescence/adolescents
- Outline reasons for focusing on Adolescents
- List types of Adolescent Sexual and Reproductive health (ASRH) problems
- Mention different types of adolescent health programs designed to address ASRH problems
- In addition, highlight the Ethiopian adolescents/youth sexual and reproductive health situation briefly

- What is

- Adolescence?

- Adolescents?

- Teenager?

- Puberty?

- Youth?

What is Adolescence?

- The period
 - when the individual progresses from the point of initial appearance of 2^o sexual characteristics to sexual maturity
 - of rapid physiological changes and vulnerability to physical, psychological and environmental influences
 - of transition from the state of total socio – economic dependency to relative independence
 - of physical, biological, psychological and social maturity from childhood to adulthood

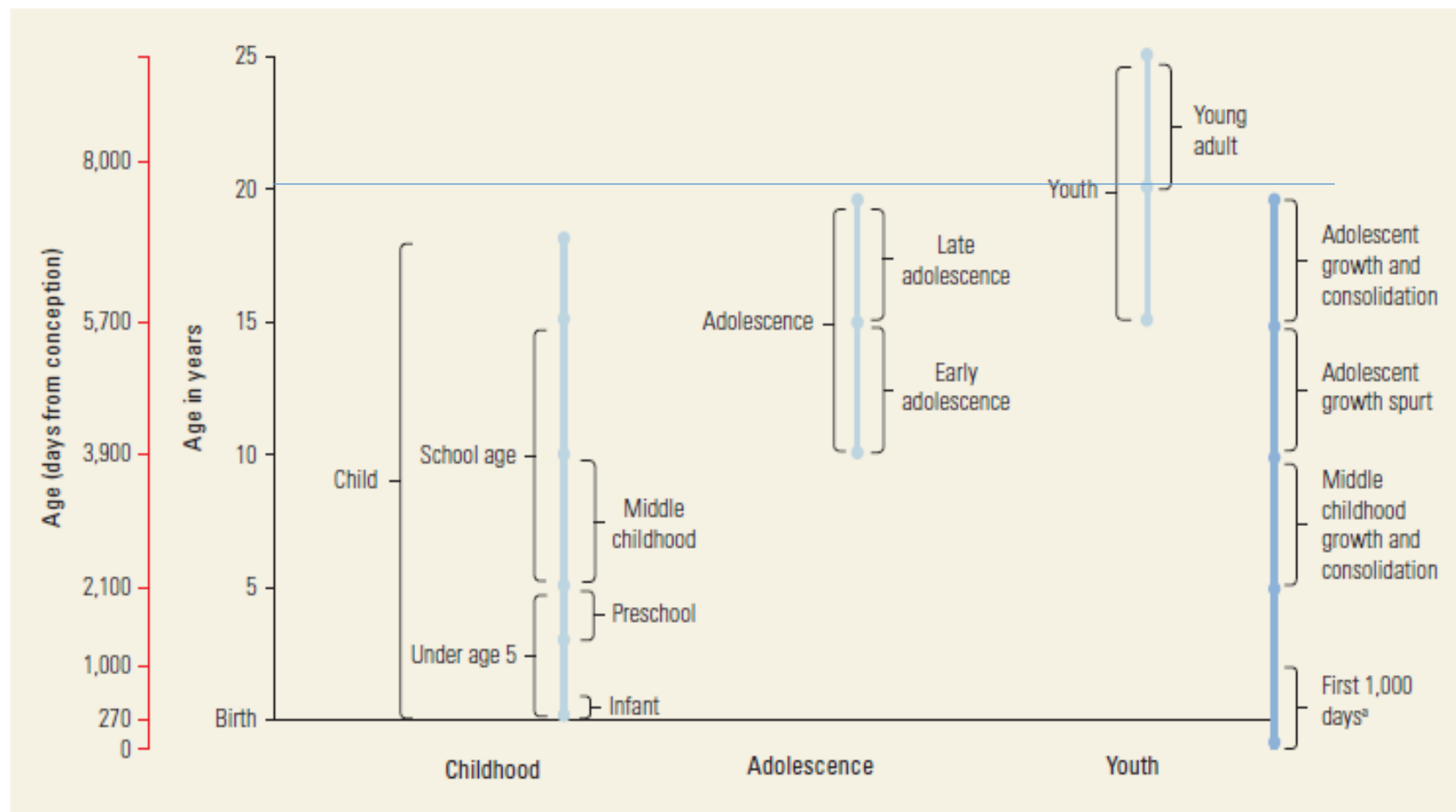
What is Adolescence?

- Generally, it is the period of dynamic change representing the transition from childhood to adulthood.
- It begins at puberty, i.e.
 - for girls, it is generally marked by the production of oestrogen, growth of breasts, the appearance of pubic hair, growth of the external genitals, and menarche
 - for boys, it begins with the production of testosterone, enlargement of the testes and penis, a deepening of the voice, and a growth spurt
- Physical maturation occurs earlier in young women than in young men, but psychological and emotional readiness for the potential consequences of sexual activity occur much later than menarche.

What is Adolescence?

- WHO classified age group
 - 10 – 19 as **adolescents**,
 - 15 – 24 as **youth** and,
 - 10 – 24 as **young people**.
- The duration and defining characteristics of adolescence may vary across *time, culture, and socio-economic* situations.

Figure 1.1 Nomenclature Concerning Age and Four Key Phases of Child and Adolescent Development



Bundy et al, 2017. Child and Adolescent Health and Development.
Disease Control Priorities (3rd edition), Volume 8. Washington, DC: World Bank.

Adolescents are a diverse population group

Different needs

Changing needs



Why Focus on Adolescents and Young Adults?

- **Huge number**

- More than 1/4th of the world's population are b/n the ages of 10 and 24
 - *In 2019 out of 7.7 Billion, 24% were young people*
 - *86% of these youth live in developing countries.*
- This # is large in SSA representing **1/3 (32%)** of the total population in 2019.
- In Ethiopia, in 2019 of projected 110 M total population, some **34%** are young people (10 – 24).

Why Focus on Adolescents and Young Adults?

- **Conducive time**

- Traditionally, conducive time for transferring knowledge, information and skills about sexuality, relationships, marriage, and parenting.
- This included
 - interaction with members of the same sex & initiation
 - extended courtship periods that served as training and
 - preparation for marriage etc...

Why Focus ...

- **Changes/Transformations**

- Over the past century, adolescence has been radically altered by
 - earlier onset of puberty
 - later age of marriage
 - urbanization
 - global communication and
 - changing sexual attitude and behaviours

- **Different Needs and Approaches**

- Adolescents have *different needs* at different stages of their development and require different approaches to reaching and serving them

Why Focus ...

- **Unmet Needs/demands**

- Existing service delivery systems have proven to be ill equipped to meet these demands
- Programs and services often failed to take into account the psychosocial cognitive differences among youth and between youth and adults

- **Existence of Barriers**

- There are real and perceived barriers that limit access and utilization of RH information and services
- Laws, customs and practices often impact adolescents differently than adults.
 - e.g. unmarried youth are denied access to information and service altogether.

- **Limited decision-making capacity**

- Both married and unmarried youth lack or have limited control over sexual decision-making... This is particularly true for young women and girls.

Why Focus ...

- **Institutional policies and attitudes**

- Provider attitude and institutional policies and procedures are often **judgemental and hostile** to young people who responsibly seek information and services.

- **Culture/Taboos**

- Taboos against discussing issues related to sexuality often leave young people uninformed or misinformed.

Adolescents Health problems

- What are the major health problems of adolescents?
- Two broad categories
 - Sexual and Reproductive Health Related Problems
 - Non-RH related Problems

Sexual and Reproductive Health Related Problems

- Adolescent/Teenage pregnancy
- Abortion and its complications
- Rape/Sexual violence/coercion
- Sexually Transmitted Infections (STIs)
- HIV/AIDS
- Harmful Traditional Practices (HTP)
 - FGM/C (Female Genital Mutilation/Cutting)
 - Early Marriage
 - Abduction, Marriage by abduction

Non-RH related Problems

- Substance use
 - Alcohol, Tobacco, Khat, other drugs – Marijuana, Cocaine,
- Injuries (intentional/unintentional) resulting from accidents or violence
 - RTAs (Road Traffic Accidents), recreations, sport
 - Homicides, fighting,
- Mental Health problems
 - Suicide, depression, eating disorders
- Medical problems
 - DM, TB, Malaria, cancers and other NCDs
- Malnutrition (under nutrition, over nutrition)

ADOLESCENT HEALTH PROBLEMS

- **Immediate Causes**

- Inadequate information and skills
- Poor access to education and health services;
- Unsafe and unsupportive environment (from family and friends; service providers; policies/legislation and the media);
- Exploitation and abuse

ADOLESCENT HEALTH PROBLEMS

- **Underlying Causes/factors**

- Poverty,
- Rapid social change and urbanization, Migration;
- Gender, ethnic, and other forms of discrimination;
- Luck/Inadequate Education
- Unemployment/Limited work opportunities;
- Social values and norms;
- Civil unrest (wars and emergencies)
- Violence
- HIV/AIDS

ADOLESCENT SRH

- ***Adolescent sexual and reproductive health (ASRH)***

refers to

the physical and emotional well-being of adolescents,
and includes

their ability to remain free from

unwanted pregnancy, unsafe abortion, STDs including
HIV/AIDS and

all forms of sexual violence and coercion.

ADOLESCENT SRH PROBLEMS

- The major causes of adolescent SRH problems are early and unprotected sexual intercourse and early and usually unwanted pregnancy

which may be due to

- 1) Lack of knowledge on physiology of the reproductive system and human sexuality
- 2) Declining age of menarche
- 3) Early marriage
- 4) Urbanization, migration, and cultural influences (western)
- 5) Sexual violence and coercion

ADOLESCENT SRH PROBLEMS

6) Peer influence/pressure

7) Lack of knowledge on contraceptive use and methods

8) Unavailability and inaccessibility of services (including cultural influences)

9) Attitude of the society including service providers towards use of FP services by adolescents

10) Sense of guilt, fear of discovery, disapproval or rejection

Adolescent Health Programs

- A WHO/UNFPA/UNICEF study group developed a framework for country programming for adolescent health.
- The framework consisted of:

Adolescent Health Programs

1. OBJECTIVES

- **To promote healthy development to meet needs**
 - Safety “belonging”
 - Self esteem
 - Caring relationships
- **To build competencies**
 - Physical e.g. eating habits
 - Psychological e.g. empathy
 - Social e.g. communication
 - Moral e.g. personal responsibility
 - Vocational e.g. entrepreneurial
- **To prevent and respond to health problems**
 - Early, unprotected and unwanted sex
 - Use of tobacco and misuse of alcohol and other substances
 - Accidents
 - Violence
 - Poor nutrition
 - Endemic diseases

Adolescent Health Programs

• **GUIDING CONCEPTS**

- Adolescence - a time of opportunities and risks
- Not all youth are equally vulnerable
- Adolescent development underlies prevention of health problems
- Problems have common roots and are interrelated
- Social environment influences adolescent behaviour
- Gender considerations are fundamental

Adolescent Health Programs

CHALLENGES

- **Building political commitment**
 - “***Young people are healthy***”, but behaviours starting during adolescence are crucial to current and future health.
 - “***The issues are too sensitive***”, but the stakes are too high to ignore and there are those able to act.
 - “***Young people make economic and political demands***”, but they are also a great resource for social and economic progress.
 - “***There is no money***” but much can be done with existing resources.

Adolescent Health Programs-challenges

- **Identifying priorities of action**
 - Collect and analyse data by age and sex
 - Compile data from different sectors
 - Involve young people systematically
 - Determine target groups
- **Maintaining implementation**
 - foster adult-youth partnership
 - reorient and sustain existing interventions through training
 - coordinate activities in multiple settings
 - recruit young people as human resources
 - achieve large scale programming

Adolescent Health Programs

☐ **Monitoring and evaluation**

- Establish and use indicators,
- Track quality and coverage in multiple settings
- Understand the impact of youth participation throughout programming

☐ **Major interventions**

- Create safe and supportive environment
- Provide information
- Build skills
- Provide counselling
- Improve health services

Adolescent Health Programs

☐ **Intervention settings**

- Home
- School
- Health institution
- Workplace
- Street
- Community organization
- Residential centre
- Media/entertainment
- Political and legislative systems

Adolescent Health Programs

❑ **Key to success**

- Put youth at the centre
- Address multiple health problems
- Build on and link existing interventions in various settings
- Combine interventions
- Respect cultural diversity
- Strengthen program management
- Encourage positive adult attitude

- What do you understand from the term **first sexual intercourse (FSI)**?
- Does this have any relevance to the health of adolescents?
- FSI in Ethiopia? Males? Females?

Age at first sexual intercourse (FSI)

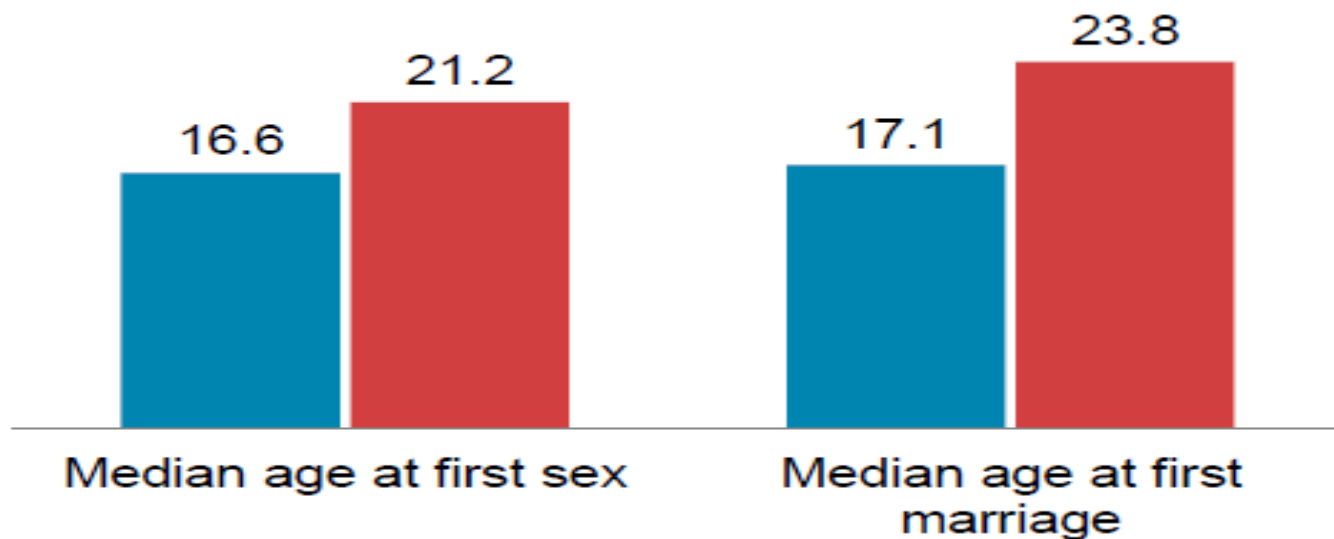
- According to EDHS 2016,
 - It indicates that both women and men engage in sex before marriage.
 - In Ethiopia, **among women age 25-49.**
 - One in four (**24%**) women have FSI before age 15, and **62%** before age 18,
 - By age 20, **76%** of women have had sexual intercourse.
 - In Ethiopia, **among men age 25 - 49.**
 - Only **2%** of men have first sex before age 15,
 - while **17%** have initiated sexual intercourse by age 18,
 - By age 20, **36%** of men have had sexual intercourse.

Ethiopian DHS, 2016

Figure 4.4 Median age at first sex and first marriage

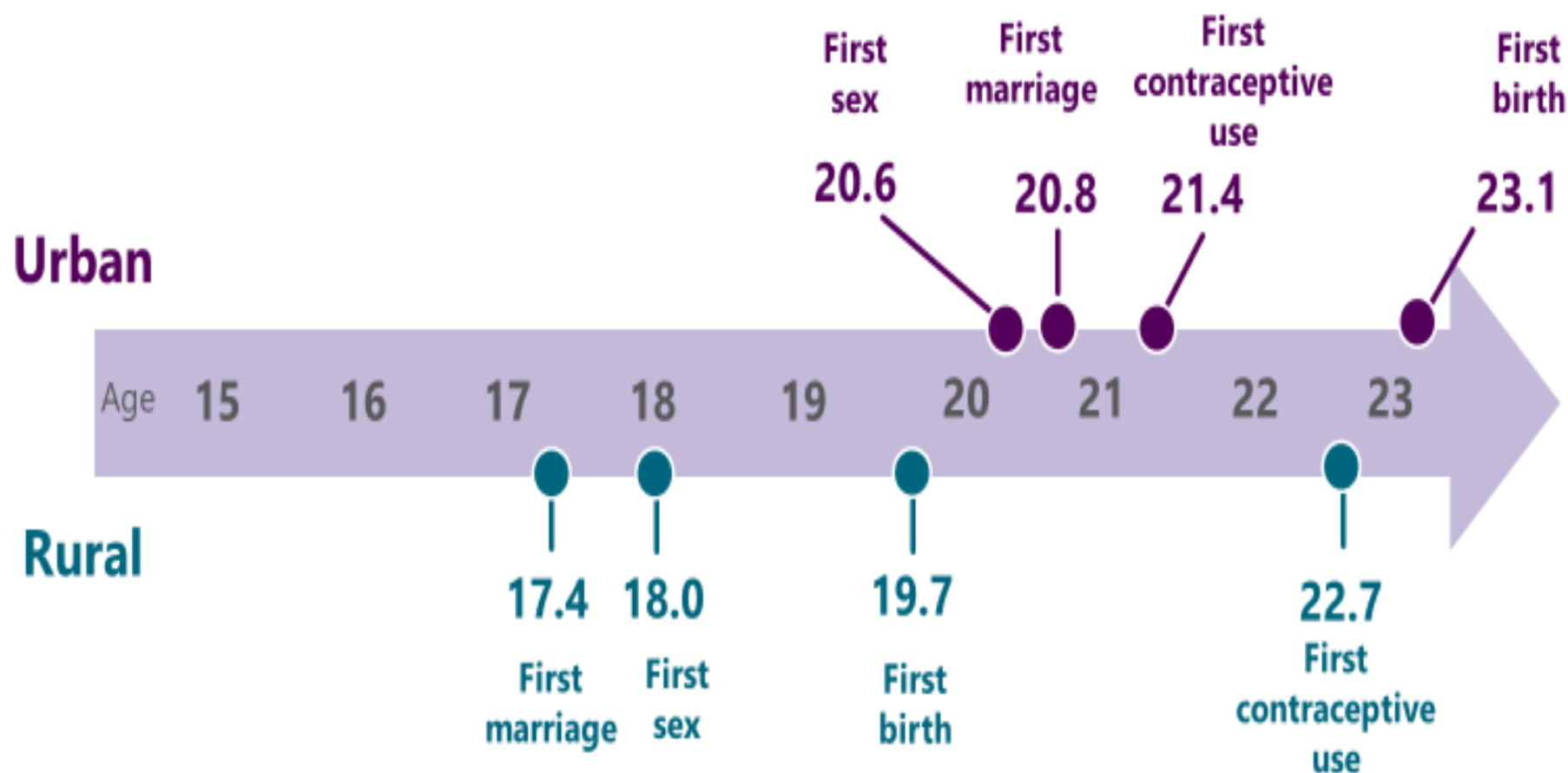
Median age in years

■ Women age 25-49 ■ Men age 25-59



Reproductive Timeline (PMA 2020)

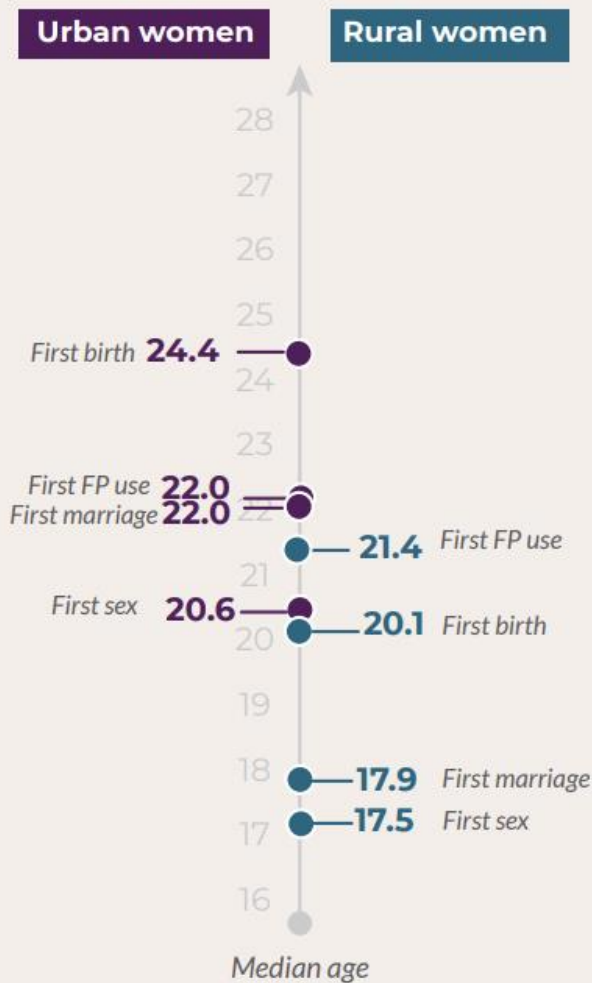
Median age for reproductive milestones, urban vs. rural women (weighted)



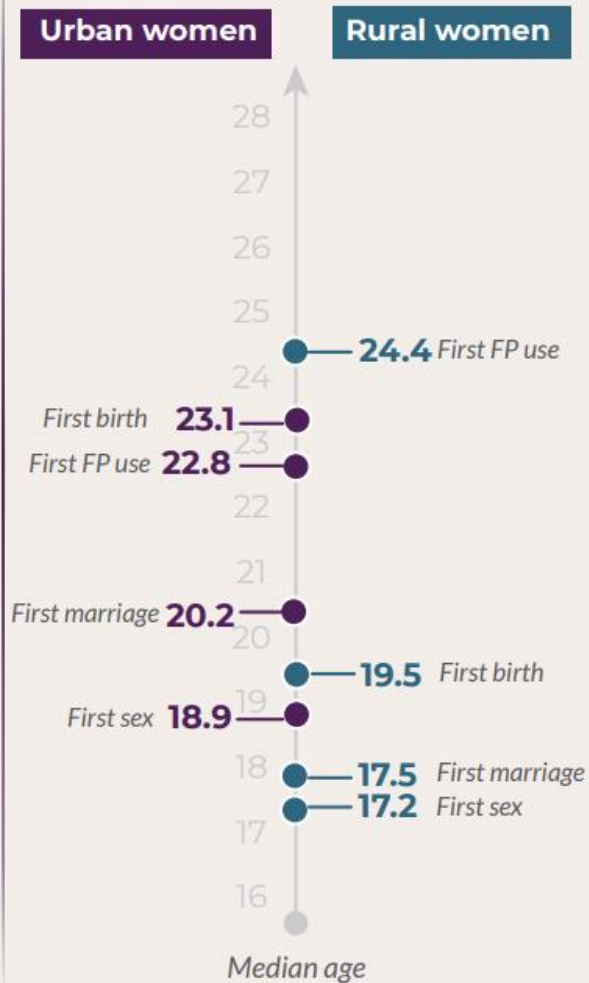
REPRODUCTIVE TIMELINE

Median age at reproductive events, by residence and age group

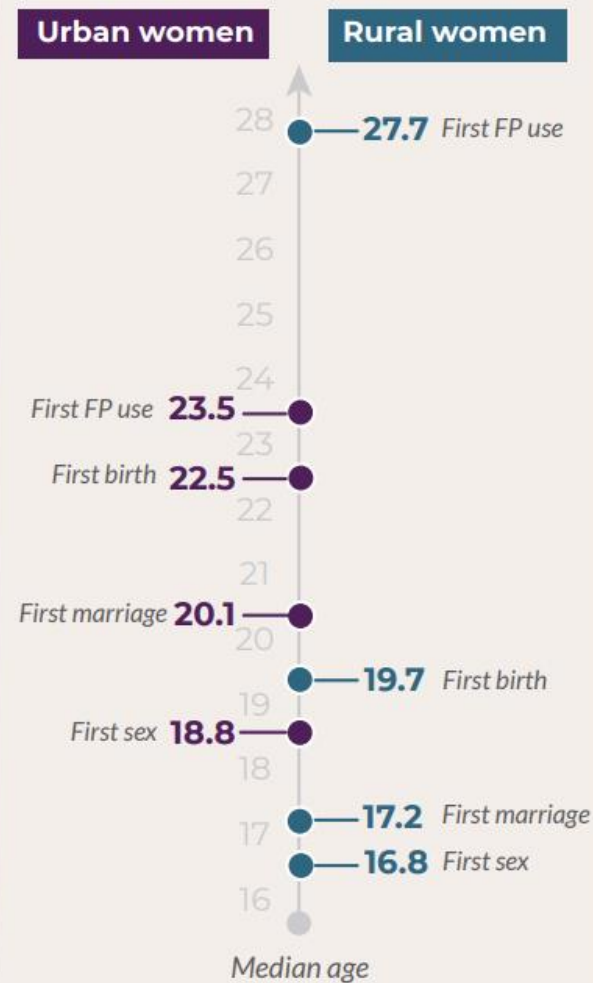
Age group: 25-29 (n=1466)



Age group: 30-34 (n=1047)



Age group: 35-39 (n=930)



PMA Ethiopia, Results from 2020 surveys,

Thank You!